

FAM Sickness and Pandemic Guide

Faded's Advanced Medical - Build 42.18 rebuild notes. Generated May 21, 2026 from the current mod Lua.

1. What the sickness number means

FAM does not treat sickness as one single disease. The displayed sickness signal is the highest of body sickness, food sickness, and the related character sickness stats.

Temperature is tracked separately as deviation from 37.0 C. Fever and hypothermia both raise the temperature-risk bar.

Cold is also separate. It comes from cold strength, catch-a-cold state, cold moodles, and low body temperature exposure.

2. Where sickness comes from

Corpse exposure: nearby corpses and dragging bodies raise Corpse Exposure and then Pathogen Load. Sanitation greatly reduces this pressure, and both values decay when you get away from corpses.

Wounds: infected wounds, dirty open wounds, deep wounds, burns, embedded glass or bullets, exhausted tourniquets, and gangrene raise local infection, sepsis, shock, or amputation risk.

Pandemic diseases: FAM can create influenza-like illness, gastroenteritis, bacterial wound fever, pneumonia-like illness, and Hollow Veil, the corpse-borne disease.

Knox infection is separate. FAM can warn about Knox patterns, but FAM antibiotics do not cure Knox infection.

3. What players should look for

Chart tab: Temperature, Cold, Corpse Exposure, Pathogen Load, Sepsis Risk, Shock Risk, Disease, Blood Reserve, and Clinical Summary are the main lines.

Patient Visual: the pulse panel changes to a poison-style state when poison, food sickness, or sickness is high enough.

Pandemic tab: active cases show disease name, load, contagion, quarantine time, and treatment time.

Hollow Veil can also cause blur and temporary deafness halos when its load is high enough.

4. Treatments and what they actually do

Sanitation is the broad anti-exposure treatment. It reduces corpse exposure, pathogen load, local infection, sepsis, and sickness; if a pandemic disease is active it also reduces

pandemic load using the disease sanitation multiplier.

Antibiotic pills reduce fake infection, gangrene, sepsis, and susceptible pandemic load. They also add poison load. The antibiotic syrette is stronger, but also adds poison and fatigue.

Flu medication reduces cold strength and helps pandemic diseases that respond to flu support. It also adds fatigue and poison.

IV fluids, blood packs, and epinephrine are support tools. They help shock, blood loss, fatigue, and collapse risk, but they do not cure the disease by themselves.

Stump care and wound care reduce local wound pressure. Field amputation is for critical limb cases such as critical gangrene or long-standing critical bullet trauma, not for ordinary fever.

Quarantine reduces spread and slowly helps load. Treat Case applies a treatment window and reduces pandemic load, but it requires clinical access.

5. Why someone can die after antibiotics

Antibiotics are not an instant save. If the patient is already in severe Knox infection, sepsis, shock, blood loss, poison, or untreated trauma, the next game update can still kill them.

Commercial antibiotics add poison while reducing infection pressure. Giving them to a critical patient without stabilizing bleeding, blood reserve, shock, and exposure can make the UI look like the antibiotic caused the death even though the fatal condition was already active.

If the screen shows Knox suspected or zombie fever, treat it as a separate lethal process. FAM antibiotics are not a Knox cure.

6. Quick triage flow

1. Inspect or Refresh the patient and read Clinical Summary first.
2. If fever or sickness is high, check Disease, Pandemic Load, Corpse Exposure, Pathogen Load, Local Infection, Sepsis Risk, Shock Risk, Blood Reserve, and Knox.
3. Move the patient away from corpses. Sanitize if Corpse Exposure, Pathogen Load, or dirty open wounds are present.
4. Stop bleeding first. Use hemostatic, tourniquet, IV fluids, or blood packs before chasing disease labels on an unstable patient.
5. Use antibiotics for bacterial wound fever, pneumonia-like disease, sepsis pressure, gangrene, or infected wounds. Use flu support for cold/influenza-like cases.
6. Quarantine active pandemic cases and recheck every in-game hour.

7. Useful thresholds from the current build

Corpse Exposure suspect: 25. Corpse-virus roll can begin around Exposure 62 and Pathogen 38 when the patient is not sanitized or immune.

Pathogen elevated: 20. Pathogen critical: 70.

Local infection suspect: 18. Confirmed: 40. Critical: 75.

Sepsis suspect: 25. Critical: 70.

Gangrene suspect: 30. Confirmed: 60. Critical: 85.

Bullet critical timing: 60 in-game hours. This can contribute to amputation indication.

8. Current UI notes

The triage panel now requests a server-side patient snapshot keyed to the viewer and viewed patient. This is meant to make the inspector see the same FAM vitals the patient sees.

Protocol manuals exist in the medical book pools and now also have low-weight fallback entries in broader book and magazine pools.